

Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 01-APR-2026

1. Administrative & Study Identification

Full Study Title:	AN INTERVENTIONAL PHASE 3, DOUBLE-BLIND, RANDOMIZED STUDY TO EVALUATE EFFICACY AND SAFETY OF PF-08634404 IN COMBINATION WITH CHEMOTHERAPY VERSUS PEMBROLIZUMAB IN COMBINATION WITH CHEMOTHERAPY IN ADULT PARTICIPANTS WITH LOCALLY ADVANCED OR METASTATIC NON-SMALL CELL LUNG CANCER
Short Title:	A Study to Learn About the Study Medicine Called PF-08634404 in Combination With Chemotherapy in Adult Participants With Locally Advanced or Metastatic Non-Small Cell Lung Cancer
Protocol Number:	73445812145816655
NCT Number:	NCT07222566
Sponsor Name:	Pfizer
Investigational Product:	pembrolizumab
Phase:	PHASE3
Medical Monitor:	[Not Provided in Posting]

2. Study Synopsis & Rationale

2.1 Study Objective

Primary Objective:	1. Overall Survival: Overall survival defined as the time from the date of randomization to the date of death due to any cause. 2. Progression Free Survival (PFS) assessed by blinded independent central review (BICR): Progression-free survival is defined as the time from the date of randomization to the date of the first documentation of objective progressive disease (PD) assessed by BICR per RECIST v1.1, or death due to any cause, whichever occurs first.
Secondary Obj.:	1. Confirmed objective response rate (ORR) using RECIST v1.1 as assessed by BICR 2. Progression Free Survival as assessed by Investigator 3. Confirmed ORR using RECIST v1.1 as assessed by investigator

2.2 Background & Rationale

This study is being done to find out if a new medicine called PF-08634404, when given with chemotherapy, works better than the present standard treatment (pembrolizumab with chemotherapy) for adults with a type of lung cancer called non-small cell lung cancer (NSCLC) that is either locally advanced (spread to nearby tissues) or has spread to other parts of the body.

3. Study Design & Methodology

Design Framework:	Type: INTERVENTIONAL, Phase: PHASE3, Status: NOT_YET_RECRUITING
Target N:	1500

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4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria

- * 18 years of age or older at screening.
- * Have pathologically confirmed locally advanced (Stage IIIB/IIIC) or metastatic (Stage IV) squamous or non-squamous NSCLC and not be a candidate for complete surgical resection and curative concurrent/sequential chemoradiotherapy (according to the 9th edition of the Union for International Cancer Control and American Joint Committee on Cancer lung cancer Tumor, lymph nodes, metastasis (TNM) staging system).
- * Have tumor tissue available, either paraffin block or slides from a core, excisional or fine needle biopsy
- * PD-L1 status available based on local testing results
- * Measurable disease based on RECIST v1.1 per investigator.
- * Eastern Cooperative Oncology Group performance status (ECOG) score of 0 or 1
- * Expected survival ≥ 12 weeks

4.2 Exclusion Criteria

- * Participants with known actionable genomic alteration (AGAs), including estimated glomerular filtration rate (EGFR), anaplastic lymphoma kinase (ALK), Repressor of Silencing 1 (ROS1), neurotrophic tyrosine receptor kinase (NTRK), v-raf murine sarcoma viral oncogene homolog B1 (BRAF), rearranged during transfection (RET), and mesenchymal-epithelial transition (MET), for which there are available first-line therapies per local standard-of-care (SOC) are ineligible. Documented negative results for EGFR, ALK, and ROS1 AGAs are required for participants with non-squamous histology.
- * Known active CNS lesions are excluded. Participants with definitively treated brain metastases (surgery and/or radiotherapy) may be eligible. Clinically inactive brain metastases of longest diameter ≤ 1 cm are permitted.
- * Participants with clinically significant risk of hemorrhage or fistula are excluded.
- * Participants with any history of another malignancy within 3 years before the first dose of study intervention, or any evidence of residual disease from a previously diagnosed malignancy.
- * Unresolved toxicities from prior anti-tumor therapy, that did not recover to NCI CTCAE v5.0 Grade 0 or 1.
- * Known to have a history of a severe allergy to any component of the study intervention, or a history of severe allergic reaction to chimeric or humanized antibody.
- * History of allogeneic organ / hematopoietic stem cell transplantation.
- * Participants with any of the following respiratory conditions:
 - * Evidence of noninfectious or drug-induced interstitial lung disease (ILD) or pneumonitis
 - * Grade ≥ 3 pulmonary disease unrelated to underlying malignancy
- * History of uncontrolled comorbidities within 6 months prior to the first dose including uncontrolled cardiac and cerebrovascular conditions, hypertension, diabetes, significant vascular disease or arterial/severe venous thromboembolic events.
- * Major surgery ≤ 4 weeks or minor surgery ≤ 3 days prior to first dose of study intervention.
- * History of severe bleeding tendency or coagulation dysfunction
- * History of esophageal varices, severe ulcers, unhealed wounds, gastrointestinal perforation, abdominal fistula, gastrointestinal obstruction, intra-abdominal abscess, or acute gastrointestinal bleeding within 6 months prior to the first dose.

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- * Participants with acute, chronic or symptomatic infections including participants positive for active HIV, hepatitis B virus (HBV), or Hepatitis C virus (HCV).
- * Participants with history of immunodeficiency
- * Any medical or psychiatric condition including recent (within the past year) or active suicidal ideation/behavior (in the past 5 years) or laboratory abnormality that may increase the risk of study participation or make the participant inappropriate for the study.
- * Previous systemic anti-tumor therapy including:
 1. Prior systemic therapy, including anti-PD-(L)1 therapy, for locally advanced, unresectable, or metastatic NSCLC.
 2. Previous treatment with immunotherapy
 3. Prior radiotherapy ≥ 30 Gy to the lung ≤ 6 months of first dose of study intervention
 4. Palliative local therapy ≤ 2 weeks before the first dose of study intervention;
 5. Non-specific immunomodulatory therapy ≤ 2 weeks before the first dose.
 6. Prior systemic anti-angiogenic therapy
- * Prior immune-related AE that led to anti-PD-(L)1 treatment discontinuation, adverse events from prior immunotherapy not improved to Grade 1 before screening, or required treatment with systemic immunosuppressive therapy.
- * Prior and concomitant therapy:
 1. therapeutic oral or parenteral anticoagulants or thrombolytic agents ≤ 10 days to the first dose.
 2. chronic antiplatelet therapy ≤ 7 days to randomization.
 3. live or attenuated live vaccine ≤ 4 weeks to the first dose.
 4. current high-dose systemic corticosteroids.
 5. prohibited concomitant medication(s) ≤ 21 days to the first dose.
- * Breastfeeding participants, participants of childbearing potential, and male participants who are unwilling to follow contraceptive measures.

11. Study Identifiers & Governance

Primary ID: 73445812145816655

Registry Number: NCT07222566

Sponsor: Pfizer

Study Title: A Study to Learn About the Study Medicine Called PF-08634404 in Combination With Chemotherapy in Adult Participants With Locally Advanced or Metastatic Non-Small Cell Lung Cancer

15. Sign-off & Approval

Principal Investigator: _____ Date: _____

Clinical Operations Lead: _____ Date: _____

Lead Statistician: _____ Date: _____